

Unmeasured pages — tranche 1

Leader lines that do not end on the structure they name

Rounds Codex · compiled 16 August 2026

HOW TO READ EVERY FIGURE IN THIS DOCUMENT

- RED** crossed circle — where the leader, dot or badge ends NOW. The number matches the table.
- GREEN** plain circle — where it belongs, with an arrow running from red to green.

Every coordinate is in the SHIPPED page's own pixels, origin top-left. Where a page is off-standard its size is printed in the page header, so the numbers can be scaled if you work from a larger master.

Where a target is given as a coordinate it was measured, and the tissue under it was sampled with the leader's own ink excluded. Where a target is described in words but not numbered, it could not be pinned to a pixel from the shipped render and needs placing from the source file — those are marked PLACE FROM SOURCE and are not guesses to be actioned blindly.

Some rows cannot be fixed by moving anything: the structure the label names is not drawn on the page. Those say so explicitly. They need artwork or a different view, and re-aiming the leader would only move the error.

WHAT IS IN HERE

5 gallery pages, 26 measured findings. These are the pages whose corrections are measured and still open. Pages already corrected and deployed are not included.

PAGE	FINDINGS	WHAT IT IS
gout-p2	9	Labeled Anatomy: First MTP Joint
hiv-p2	8	Anatomy & Viral Structure
hyperparathyroid-p2	3	Parathyroid Anatomy & Normal Physiology
gi-bleed-p1	3	Anatomy & Bleeding Sources
diverticulitis-p2	3	Normal Colon Anatomy & Diverticula

How to make the corrections

1. Moving a leader tip or a dot — most of this document

Erase the last segment along the LEADER'S OWN AXIS, then redraw to the green point. Two things keep it invisible: take the stroke width and colour by sampling the leader itself a little way back from the tip, and erase along the axis rather than with a box, so the surrounding artwork is never touched.

The test that the move is clean: at the OLD endpoint, ink in a 13x13 box should fall to ZERO. If ink remains there, the leader was extended rather than moved, and the page now carries two terminators.

2. Never repaint artwork

If a fix would mean painting over illustration - erasing a disc of shaded tissue, rebuilding a background, redrawing a structure - stop and mark the page for a re-render instead. This has been attempted here and it does not repair cleanly; every large tissue-side erase left a visible patch. Several rows in this document say exactly that and are addressed to the illustrator, not to Photoshop.

3. Rows that no move can fix

Where the structure the label names is NOT DRAWN on the page, re-aiming the leader only moves the error somewhere else. Those rows say so in the text and need artwork or a different view - for example a lateral inset, an added slab, or a redrawn tracing. Rows marked PLACE FROM SOURCE are ones we could not pin to a pixel from the shipped render; place them from the layered file.

4. Save at the SAME pixel dimensions

Do not resample, resize, crop or rotate. The coordinates in this document are in the shipped page's own pixels, and the app, the thumbnails and the download PDF are all built from that geometry. Save as JPEG at high quality - we re-encode on integration at the file's own quantisation tables and chroma sampling, so an exact quality match on your side is not needed, but a resize breaks everything downstream.

5. Send the corrected page back; do not rebuild anything else

We regenerate the thumbnail and rebuild that gallery's download PDF. That second step matters: the download PDF is a separate build artefact and drifts from the pages if it is not rebuilt - four galleries shipped for months with an old logo in the PDF while the app showed the current one.

6. One page at a time is fine

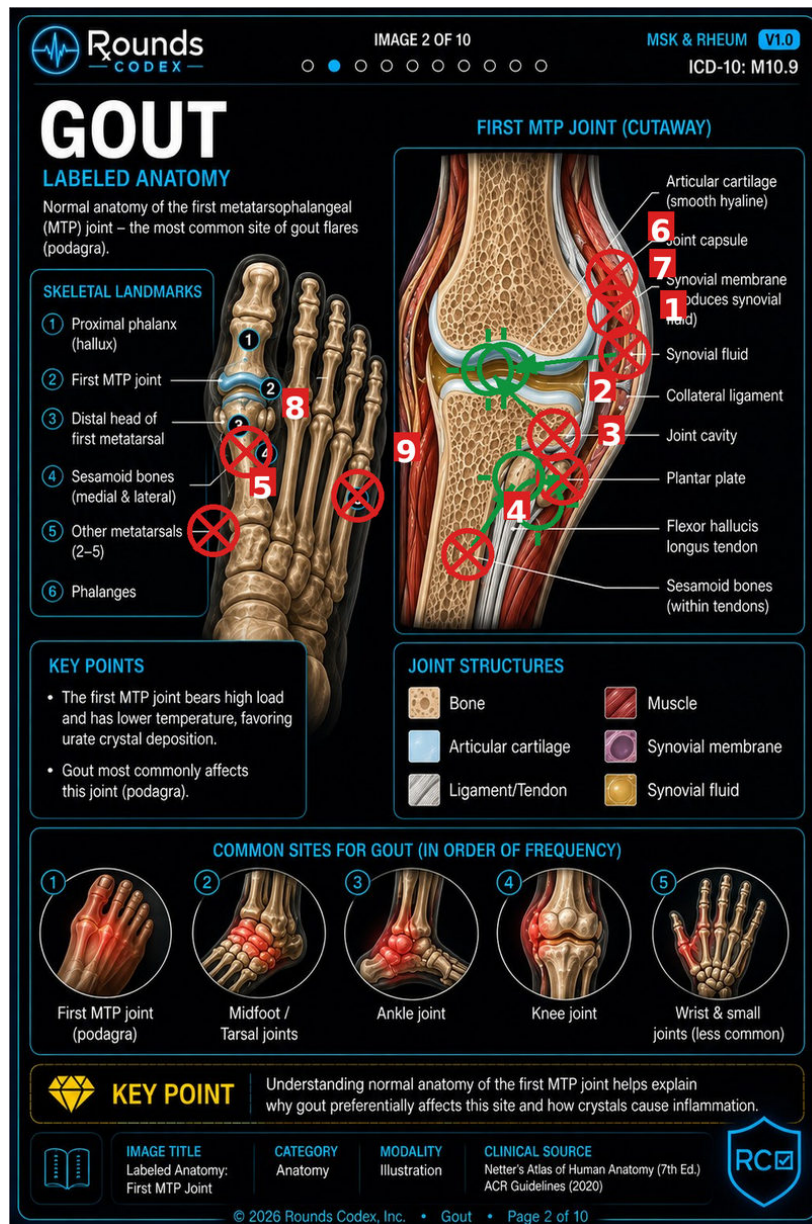
Corrections are integrated and deployed in batches of about six pages. Each one is measured against the sheet before it ships - what was fixed, what was missed, and what was left - and that measurement is reported back honestly rather than assumed.

gout-p2

Labeled Anatomy: First MTP Joint · gout-02.jpg · 1024×1536 px · 9 findings

9 of 15 labels flagged — 5 to move · 4 to check — endpoints VERIFIED by us; targets are the order's words unless a green ring is shown

Every red marker below sits where we CONFIRMED a leader terminator by counting ink in a 13x13 box at the order's stated coordinate. A green ring appears only where we measured the target ourselves; where there is no green ring the target is the work order's description and has not been verified.

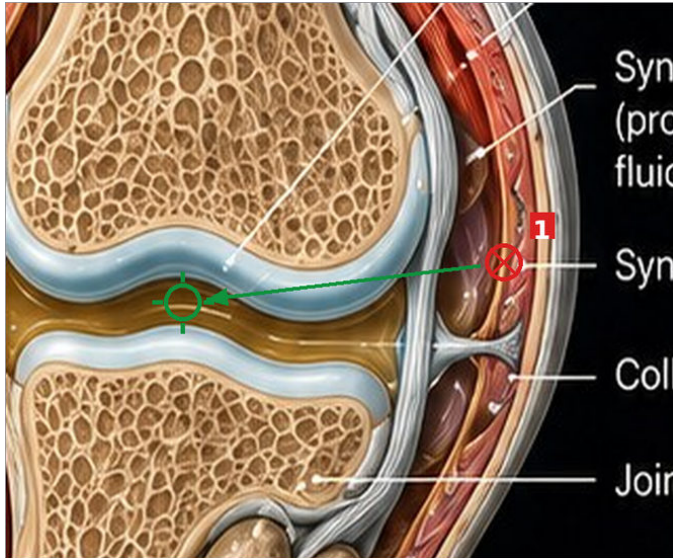


WHAT TO CHANGE

- 1 Synovial fluid**
NOW (772,434) extracapsular soft tissue / subcutaneous layer at the lateral margin of the cutaway (~x772,y434)
MOVE TO (620,453) the same gold fluid band at the centre of the cutaway
- 2 Joint cavity**
NOW (683,537) trabecular bone inside the proximal phalanx (~x683,y537), well distal to the joint
MOVE TO (600,455) the gold synovial space between the cartilage caps, sampled (104,67,17)
- 3 Plantar plate**
NOW (694,592) the lateral sesamoid bone (~x694,y592), which the page labels separately
MOVE TO (663,624) the fibrocartilage deep to the sesamoids; pale to y~616, dark at y 624-632
- 4 Sesamoid bones (within tendons)**
NOW (575,687) trabecular bone inside the proximal phalanx shaft (~x575,y687)
MOVE TO (639,590) the medial sesamoid — the two tan ovals run x 631-647 and x 679-695 at y 590
- 5 Other metatarsals (2-5) (left skeletal-landmarks panel, item 5)**
NOW (256,657) empty black background at the edge of the legend box (~x256,y657); there is no numbered 5 badge anywhere on the foot
MOVE TO metatarsals 2-5 on the lateral side of the foot
- 6 Joint capsule [CHECK]**
NOW (756,336) red muscle fibres ~15px lateral to the pale grey capsule band (~x756,y336)
MOVE TO the pale capsule band hugging the metatarsal neck
- 7 Synovial membrane (produces synovial fluid) [CHECK]**
NOW (758,382) extracapsular tissue at the muscle/capsule junction (~x758,y382)
MOVE TO the thin lining on the inner aspect of the capsule
- 8 Sesamoid bones (medial & lateral) (left panel, item 4) [CHECK]**
NOW (297,558) the first metatarsal diaphysis (~x297,y558); the 4 badge sits distal-lateral to the sesamoid pair
MOVE TO the two small ovals overlying the metatarsal head

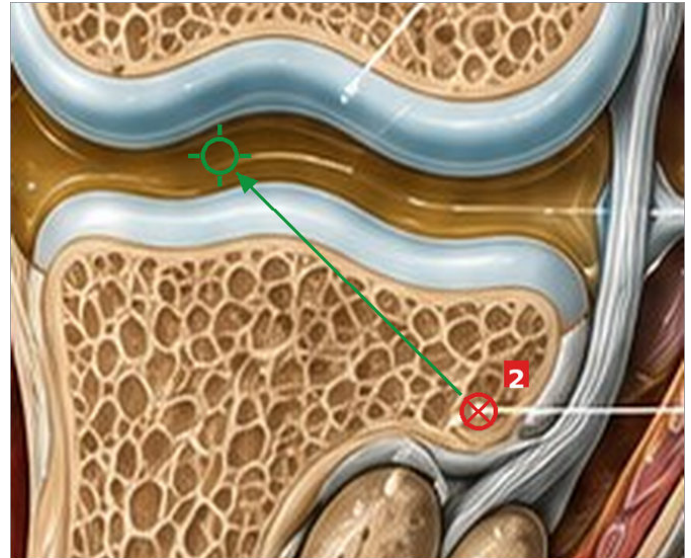
gout-p2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



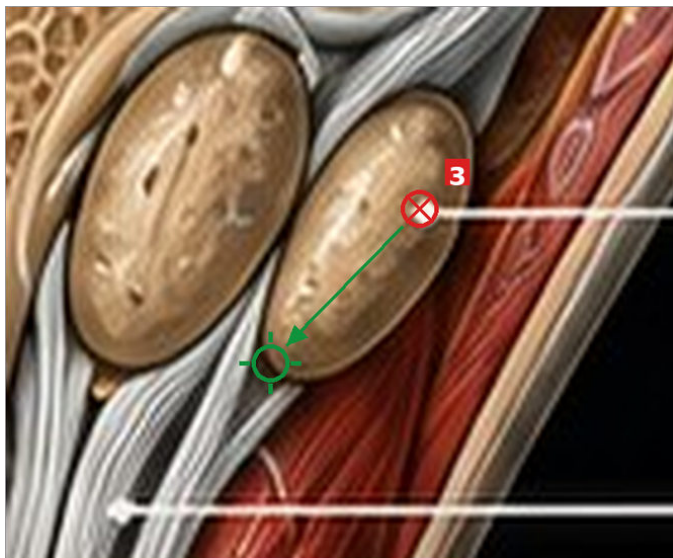
1 Synovial fluid

(772,434) extracapsular soft tissue / subcutaneous layer at the lateral margin of the cutaway (~x772,y434)
-> (620,453) the same gold fluid band at the centre of the cutaway



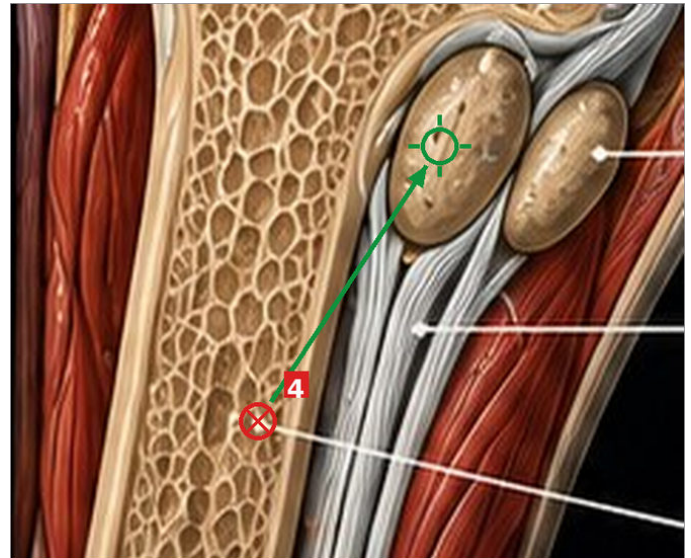
2 Joint cavity

(683,537) trabecular bone inside the proximal phalanx (~x683,y537), well distal to the joint
-> (600,455) the gold synovial space between the cartilage caps, sampled (104,67,17)



3 Plantar plate

(694,592) the lateral sesamoid bone (~x694,y592), which the page labels separately
-> (663,624) the fibrocartilage deep to the sesamoids; pale to y~616, dark at y 624-632



4 Sesamoid bones (within tendons)

(575,687) trabecular bone inside the proximal phalanx shaft (~x575,y687)
-> (639,590) the medial sesamoid — the two tan ovals run x 631-647 and x 679-695 at y 590

gout-p2 — close-ups

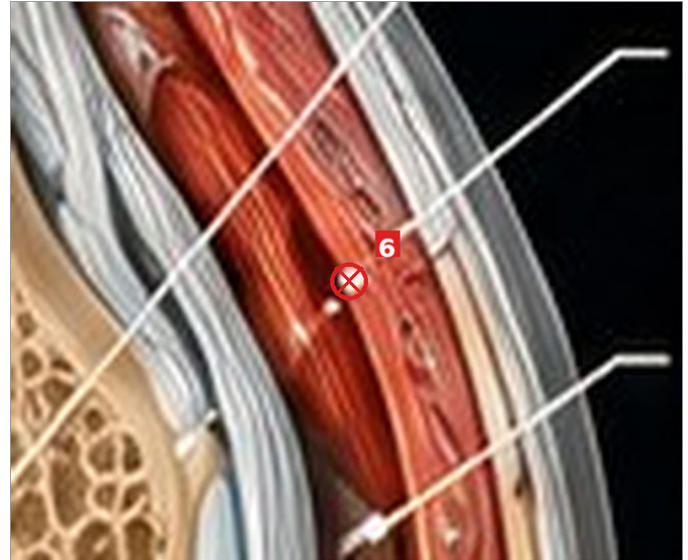
red crossed circle = where it ends now green plain circle = where it belongs



5 Other metatarsals (2-5) (left skeletal-landmarks panel, item 5)

(256,657) empty black background at the edge of the legend box (~x256,y657); there is no numbered 5 badge anywhere on the foot

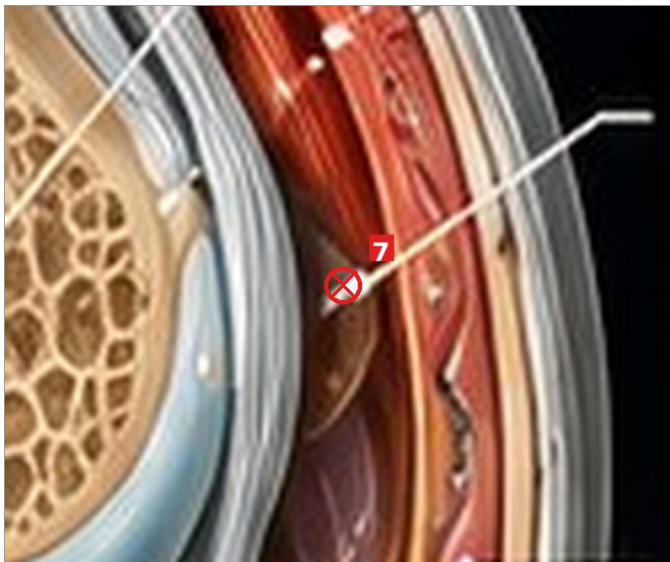
-> metatarsals 2-5 on the lateral side of the foot



6 Joint capsule [CHECK]

(756,336) red muscle fibres ~15px lateral to the pale grey capsule band (~x756,y336)

-> the pale capsule band hugging the metatarsal neck



7 Synovial membrane (produces synovial fluid) [CHECK]

(758,382) extracapsular tissue at the muscle/capsule junction (~x758,y382)

-> the thin lining on the inner aspect of the capsule



8 Sesamoid bones (medial & lateral) (left panel, item 4) [CHECK]

(297,558) the first metatarsal diaphysis (~x297,y558); the 4 badge sits distal-lateral to the sesamoid pair

-> the two small ovals overlying the metatarsal head

gout-p2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



9 Phalanges (left panel, item 6) [CHECK]

(437,613) the 5th metatarsal shaft just proximal to the
5th MTP joint (~x437,y613)

-> the phalanges of the 5th toe, one bone distal

NOTES

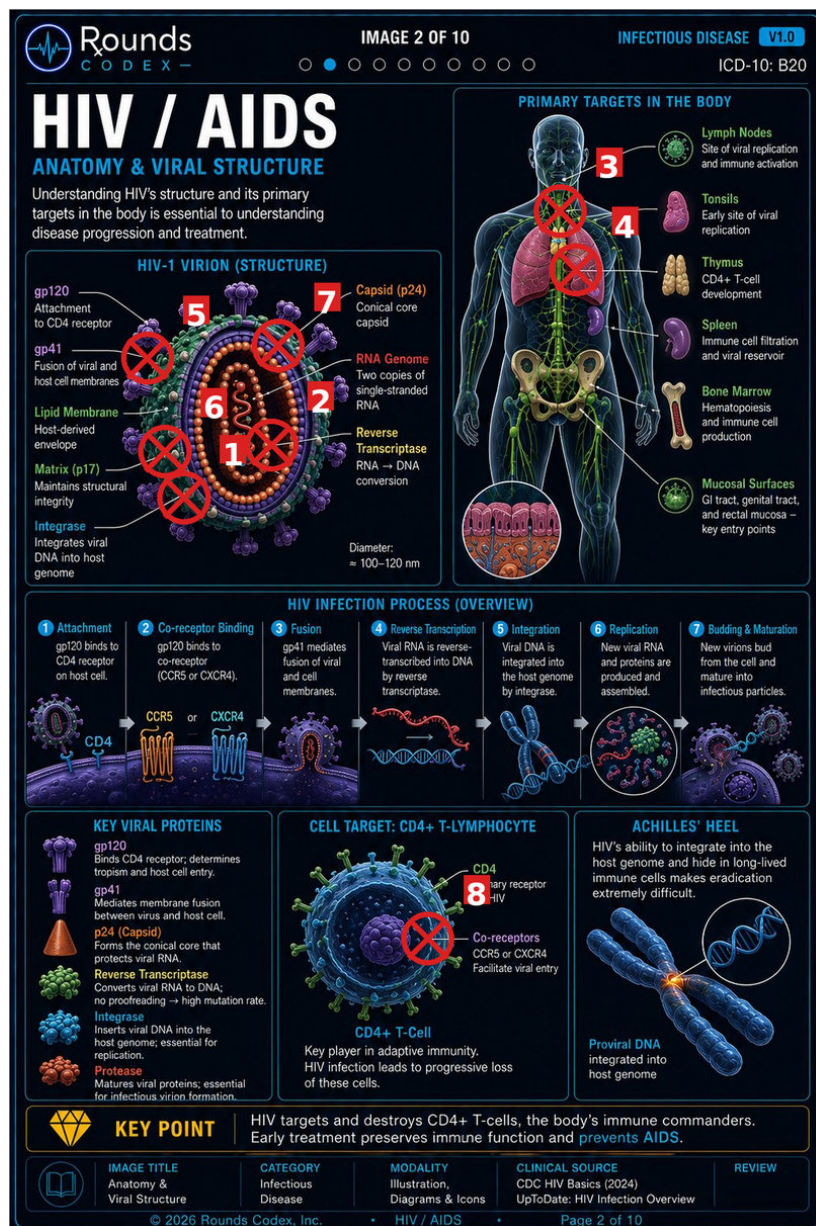
Every one of the order's nine coordinates was checked and eight carry real leader ink (15-51 px in a 13x13 box). Row 5 is the exception and confirms the order: it samples (14,18,21), near-black, i.e. the empty background the order describes - there is no numbered 5 badge on the foot at all. Rows 3 and 4 are a displaced pair: Plantar plate currently ends ON a sesamoid, and Sesamoid bones ends in the phalangeal trabecular bone.

hiv-p2

Anatomy & Viral Structure · hiv-02.jpg · 1024×1536 px · 8 findings

8 of 17 labels flagged — 4 to move · 4 to check — endpoints VERIFIED by us; targets are the order's words unless a green ring is shown

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WHAT TO CHANGE

1 Integrase (HIV-1 VIRION panel)

NOW (219,613) a beige/grey surface-protein knob on the outer green envelope (~x219,y613)

MOVE TO the cyan enzyme blob inside the capsid (~x299,y559), matching the blue integrase icon in the KEY VIRAL PROTEINS legend

2 Reverse Transcriptase (VIRION)

NOW (329,547) an orange p24 capsid bead of the inner shell (~x329,y547)

MOVE TO one of the small green enzyme shapes inside the core (~x300,y514)

3 Tonsils (PRIMARY TARGETS panel)

NOW (691,253) the thyroid / upper trachea at the thoracic inlet (~x691,y253)

MOVE TO the oropharynx, ~50px higher, behind the mouth

4 Thymus (PRIMARY TARGETS)

NOW (711,327) inside the patient's left lung (~x711,y327)

MOVE TO the yellow bilobed thymus between the lungs (~x680-698,y273-307)

5 gp41 (VIRION) [CHECK]

NOW (173,438) empty black background just outside the envelope, between spikes (~x173,y438)

MOVE TO the purple stalk of a spike, where gp41 sits

6 Matrix (p17) (VIRION) [CHECK]

NOW (195,555) the green envelope surface (~x195,y555), the same layer "Lipid Membrane" points at

MOVE TO the violet bead ring just inside the envelope

7 Capsid (p24) (VIRION) [CHECK]

NOW (337,422) the violet inner-membrane band (~x337,y422), just outside the orange p24 bead shell

MOVE TO the orange bead shell or the conical core

8 Co-receptors (CELL TARGET: CD4+ T-LYMPHOCYTE) [CHECK]

NOW (525,1167) cytoplasm just inside the membrane (~x525,y1167), overshooting the teal membrane knobs its shaft passes between

MOVE TO one of the teal co-receptor knobs on the membrane

hiv-p2 — close-ups

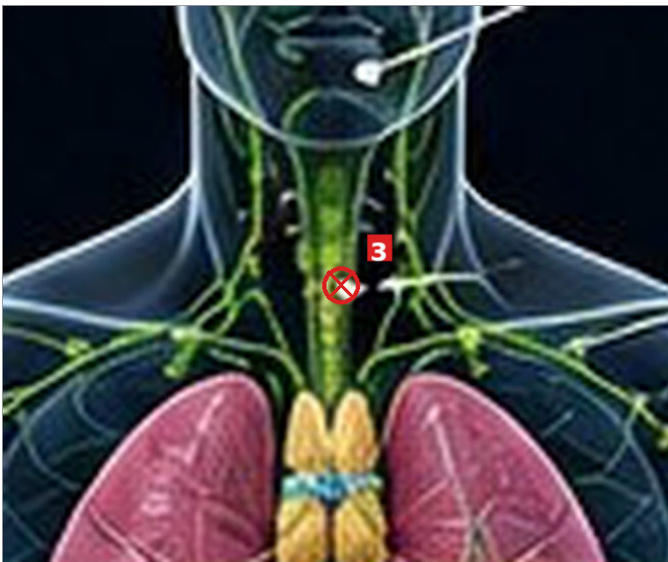
red crossed circle = where it ends now green plain circle = where it belongs



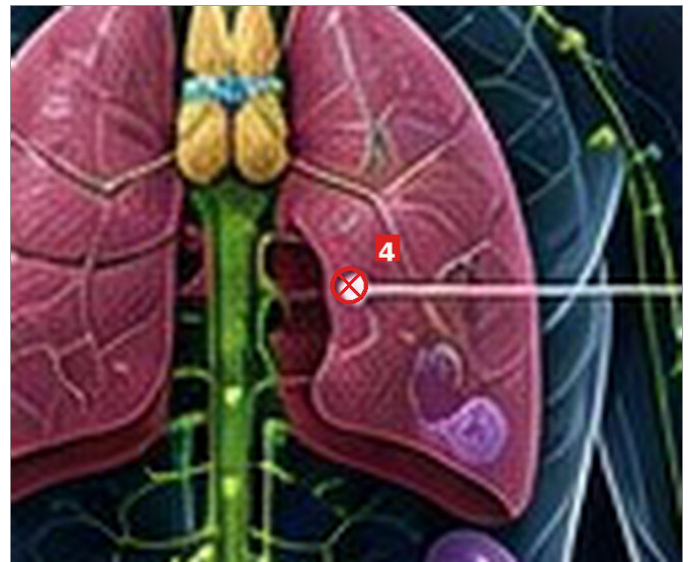
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 -> the cyan enzyme blob inside the capsid (~x299,y559), matching the blue integrase icon in the KEY VIRAL PROTEINS legend



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 (329,547) an orange p24 capsid bead of the inner shell (~x329,y547)
 -> one of the small green enzyme shapes inside the core (~x300,y514)



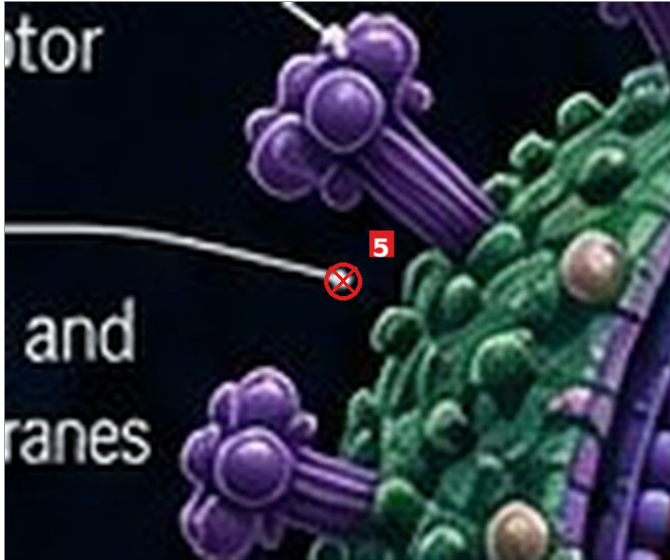
- 3 Tonsils (PRIMARY TARGETS panel)**
 (691,253) the thyroid / upper trachea at the thoracic inlet (~x691,y253)
 -> the oropharynx, ~50px higher, behind the mouth



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 (711,327) inside the patient's left lung (~x711,y327)
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hiv-p2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



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(195,555) the green envelope surface (~x195,y555), the same layer "Lipid Membrane" points at
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(337,422) the violet inner-membrane band (~x337,y422), just outside the orange p24 bead shell
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(525,1167) cytoplasm just inside the membrane (~x525,y1167), overshooting the teal membrane knobs its shaft passes between
-> one of the teal co-receptor knobs on the membrane

NOTES

All eight of the order's coordinates carry leader ink (8-23 px). No target has been measured on this page yet - the descriptions are the order's.

hyperparathyroid-p2

Parathyroid Anatomy & Normal Physiology · hyperparathyroid-02.jpg · 1024×1536 px · 3 findings

3 of 5 labels flagged — 2 to move · 1 to check — endpoints VERIFIED by us; targets are the order's words unless a green ring is shown

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Rounds CODEX — IMAGE 2 OF 10 ENDOCRINE v1.0 ICD-10 E21.3

HYPERPARATHYROIDISM

PARATHYROID ANATOMY & NORMAL PHYSIOLOGY

“The parathyroid glands sense serum calcium and regulate calcium, phosphate, and vitamin D balance through PTH.”

KEY POINT

- Four small glands on the posterior thyroid.
- Chief cells secrete PTH when Ca^{2+} is low.
- PTH acts on bone, kidney, and gut to raise serum calcium.

PARATHYROID GLANDS: LOCATION & ANATOMY

Posterior View

Superior parathyroid glands (2)

Inferior parathyroid glands (2)

Typically 4 glands (2 superior, 2 inferior). Variations in number, size, and location are common.

ANATOMIC VARIATIONS

- Ectopic: In mediastinum
- Intrathyroidal: Within thyroid tissue
- Supernumerary: More than four glands
- Asymmetric: Variable size and location

PARATHYROID HISTOLOGY

- Chief Cells (~80%)
 - Small, basophilic cells that secrete PTH.
- Oxyphil Cells (~20%)
 - Larger, eosinophilic cells with unclear function.
- Adipose Tissue
 - Fat increases with age.

NORMAL PTH PHYSIOLOGY: MAINTAINING CALCIUM HOMEOSTASIS

- 1. TRIGGER**
↓ Serum Calcium (hypocalcemia)
Detected by Calcium-Sensing Receptor (CaSR) on chief cells
- 2. PTH SECRETION**
↑ PTH release from chief cells
- 3. TARGET ORGANS & EFFECTS**
 - Bone:** ↑ Osteoclast activity (bone resorption) → ↑ Ca^{2+} release, ↑ PO_4^{3-} release
 - Kidney:** ↑ Ca^{2+} reabsorption (distal tubule), ↓ PO_4^{3-} reabsorption (proximal tubule), ↑ 1 α -hydroxylase → ↑ 1,25-(OH) $_2$ D
 - Gut:** ↑ Calcium absorption (via 1,25-(OH) $_2$ D), ↑ Phosphate absorption
- 4. OUTCOME**
↑ Serum Calcium Normalization

NORMAL FEEDBACK: Rising serum calcium suppresses PTH secretion.

PTH REGULATION

Stimulators (increase PTH)

- ↑ Serum calcium
- ↑ Serum phosphate
- ↓ 1,25-(OH) $_2$ D

Inhibitors (decrease PTH)

- ↑ Serum calcium
- ↓ 1,25-(OH) $_2$ D

CaSR ON CHIEF CELL

Extracellular Ca^{2+}

Low Ca^{2+} → Increase PTH

High Ca^{2+} → Decrease PTH

PTH QUICK FACTS

- PTH is a peptide hormone (84 amino acids).
- Secreted within minutes of low serum calcium.
- Acts on kidney within minutes; on bone within hours.
- Works with calcitriol (1,25-(OH) $_2$ D) to increase gut calcium absorption.
- Too much PTH → hypercalcemia (primary disease).
- Too little PTH → hypocalcemia (hypoparathyroidism).

KEY POINT

- Four glands on the posterior thyroid (2 superior, 2 inferior).
- Chief cells sense calcium via CaSR and secrete PTH.
- PTH raises calcium by acting on bone, kidney, and gut.
- Calcium feedback loop tightly regulates PTH secretion.

MODULE TITLE: Hyperparathyroidism Anatomy & Physiology

CATEGORY: Endocrine

DIFFICULTY: Intermediate

CLINICAL SIGNATURE: Hypercalcemia with high or inappropriately normal PTH

REVIEW

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WHAT TO CHANGE

- 1 Superior parathyroid glands (2)**
NOW (203,518) both of the glands on the LEFT lobe - the upper one (correct) and the LOWER one (~x203,y518), which is an inferior gland
MOVE TO the two upper glands, one on each lobe
- 2 Inferior parathyroid glands (2)**
NOW (330,518) both of its leaders converge on the same single lower-right gland (~x330,y518); the upper-right gland gets no leader at all
MOVE TO the two lower glands, one on each lobe
- 3 Adipose Tissue (PARATHYROID HISTOLOGY) [CHECK]**
NOW (611,682) parathyroid cells ~7px above the nearest fat vacuole (~x611,y682)
MOVE TO inside one of the pale yellow fat globules

hyperparathyroid-p2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



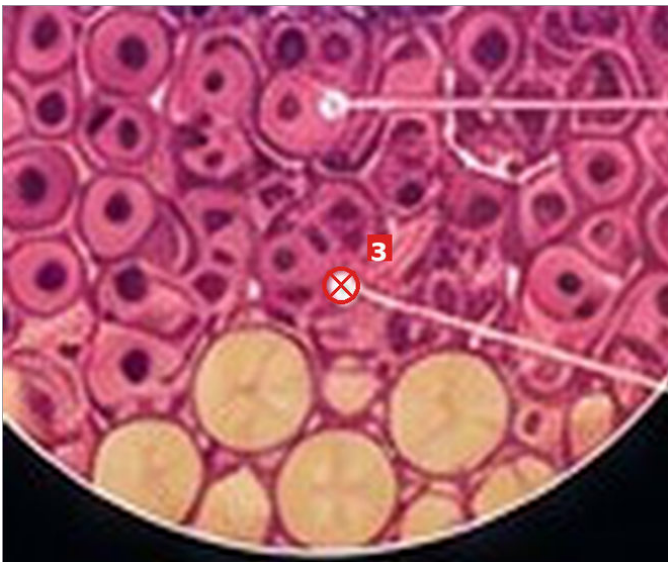
1 Superior parathyroid glands (2)

(203,518) both of the glands on the LEFT lobe - the upper one (correct) and the LOWER one (~x203,y518), which is an inferior gland
-> the two upper glands, one on each lobe



2 Inferior parathyroid glands (2)

(330,518) both of its leaders converge on the same single lower-right gland (~x330,y518); the upper-right gland gets no leader at all
-> the two lower glands, one on each lobe



3 Adipose Tissue (PARATHYROID HISTOLOGY) [CHECK]

(611,682) parathyroid cells ~7px above the nearest fat vacuole (~x611,y682)
-> inside one of the pale yellow fat globules

hyperparathyroid-p2 — notes

NOTES

Rows 1 and 2 describe GLAND POSITIONS rather than leader terminators, which is why the ink counts there are low (3 and 5) - that is expected, not a miss. Row 3 carries 22 px of ink at the stated point.

gi-bleed-p1

Anatomy & Bleeding Sources · gi-bleed-01.jpg · 1024×1536 px · 3 findings

3 of 8 labels flagged — 1 to move · 2 to check — endpoints VERIFIED by us; targets are the order's words unless a green ring is shown

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Roundscodex

IMAGE 1 OF 10

GASTROENTEROLOGY V1.0

Upper GI Bleed (ICD-10 K92.2)

UPPER GI BLEED

ANATOMY & BLEEDING SOURCES

Bleeding proximal to the ligament of Treitz – a potentially life-threatening emergency.

Esophagus

Stomach (Gastric mucosa)

Duodenum (First part)

Ligament of Treitz (Anatomical landmark separating upper GI from lower GI tract)

COMMON SOURCES OF UPPER GI BLEEDING

- Peptic Ulcer Disease (PUD)**
Gastric or duodenal ulcers due to *H. pylori* infection or NSAID/aspirin use. Most common cause.
- Esophagogastric Varices**
Portal hypertension in cirrhosis causes variceal dilation and risk of massive bleeding.
- Mallory-Weiss Tear**
Mucosal laceration at the gastroesophageal junction from forceful vomiting or retching.
- Erosive Gastritis / Esophagitis**
Mucosal irritation from NSAIDs, alcohol, or acid reflux.
- Dieulafoy Lesion**
Large, tortuous submucosal vessel causing arterial bleeding.
- Malignancy**
Gastric or esophageal cancer can present with bleeding.

Blood in the GI tract is a cathartic and nitrogen load → ↑ BUN and, in cirrhotics, may precipitate hepatic encephalopathy.

KEY POINTS

- Arises proximal to the ligament of Treitz (esophagus, stomach, duodenum).
- Presents with hematemesis, coffee-ground emesis, or melena.
- Divided into non-variceal (most common) and variceal etiologies.
- Peptic ulcer disease (*H. pylori*, NSAIDs/aspirin) is the leading cause.
- Rapid recognition and resuscitation save lives.

ANATOMY OVERVIEW

Esophagus (Mucosa)

Stomach (Gastric mucosa)

Duodenum (Up to ligament of Treitz)

Ligament of Treitz

WHO IS AT RISK?

- H. pylori* infection
- NSAIDs / Aspirin use
- Liver cirrhosis / Portal hypertension
- Alcohol use
- Advanced age
- Anticoagulant / Antiplatelet therapy

TERMS TO KNOW

Hematemesis
Vomiting of bright-red blood or material that looks like coffee grounds.

Melena
Black, tarry stools from digested blood in the intestinal tract.

Ligament of Treitz
Anatomical landmark separating upper GI (from esophagus to duodenum) from lower GI.

Non-variceal Bleed
Bleeding not related to varices; usually due to ulcers, erosions, or tears.

Variceal Bleed
Bleeding from dilated veins due to portal hypertension; often more severe.

MODULE TITLE

Upper GI Bleed Foundations

CATEGORY

Gastroenterology

DIFFICULTY

Intermediate

CLINICAL IMPACT

High

REVIEW

Anatomy & Sources Overview

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WHAT TO CHANGE

- Duodenum (First part) (main anatomy figure)**
NOW (167,674) the yellow lobulated pancreas / omental fat beside a blue vein, ~page(167,674)
MOVE TO the pink duodenal tube ~16px to the left; and the FIRST part (bulb) sits higher still, at ~page(180,570) just beyond the pylorus
- Ligament of Treitz (main anatomy figure) [CHECK]**
NOW (311,696) the smooth inferior wall of the stomach (antrum/greater curvature), ~page(311,696)
MOVE TO the duodenojejunal flexure; no suspensory ligament is actually drawn, so the dot sits on gastric wall
- Duodenum (Up to ligament of Treitz) (ANATOMY OVERVIEW panel) [CHECK]**
NOW (489,1083) the lower-right contour of the STOMACH, ~page(489,1083)
MOVE TO the duodenal loop below it, which starts ~10px lower (the "Ligament of Treitz" leader in the same panel does reach it correctly)

gi-bleed-p1 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



- 1 Duodenum (First part) (main anatomy figure)**
(167,674) the yellow lobulated pancreas / omental fat beside a blue vein, ~page(167,674)
-> the pink duodenal tube ~16px to the left; and the FIRST part (bulb) sits higher still, at ~page(180,570) just beyond the pylorus



- 2 Ligament of Treitz (main anatomy figure)**
[CHECK]
(311,696) the smooth inferior wall of the stomach (antrum/greater curvature), ~page(311,696)
-> the duodenojejunal flexure; no suspensory ligament is actually drawn, so the dot sits on gastric wall



- 3 Duodenum (Up to ligament of Treitz) (ANATOMY OVERVIEW panel) [CHECK]**
(489,1083) the lower-right contour of the STOMACH, ~page(489,1083)
-> the duodenal loop below it, which starts ~10px lower (the "Ligament of Treitz" leader in the same panel does reach it correctly)

gi-bleed-p1 — notes

NOTES

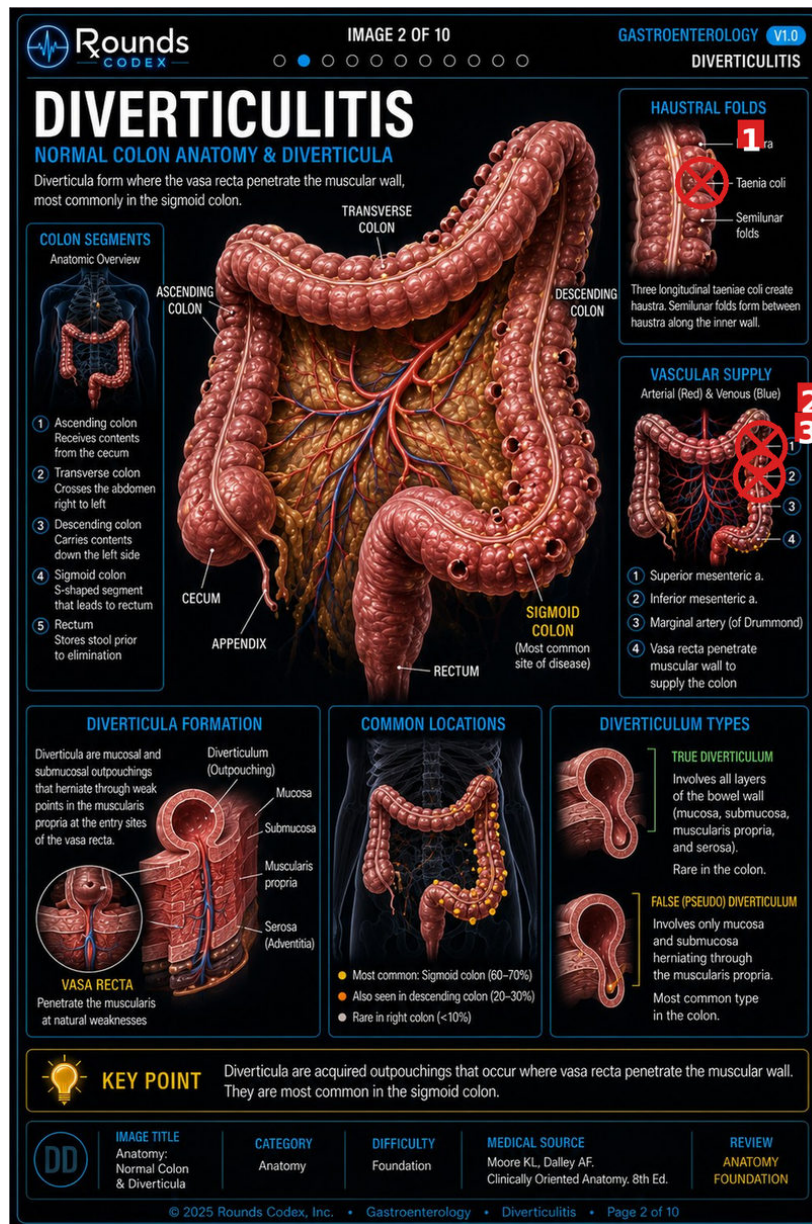
Rows 1 and 3 carry leader ink (11 and 20 px). ROW 2 IS THE ONE EXCEPTION IN THIS TRANCHE: no ink at the stated (311,696), and the tissue there samples (222,179,23), a saturated gold. Either the coordinate is a few pixels off or the leader is too thin to register - it needs measuring before it is actioned.

diverticulitis-p2

Normal Colon Anatomy & Diverticula · diverticulitis-02.jpg · 1024×1536 px · 3 findings

4 of 19 labels flagged — 2 to move · 2 to check — endpoints VERIFIED by us; targets are the order's words unless a green ring is shown

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WHAT TO CHANGE

1 Taenia coli (HAUSTRAL FOLDS panel)

NOW (869,220) the surface of a haustral sacculation, ~page(869,220)

MOVE TO the pale longitudinal band running vertically at ~page(834,220), about 35px to the left - the dot for "Haustra" above it is on the same kind of bulge

2 1 - Superior mesenteric a. (VASCULAR SUPPLY panel)

NOW (944,551) the pale longitudinal band along the DESCENDING colon, ~page(944,551)

MOVE TO the central red arterial trunk supplying the right colon; the descending colon is inferior-mesenteric territory, so the leader is on the wrong side as well as the wrong structure

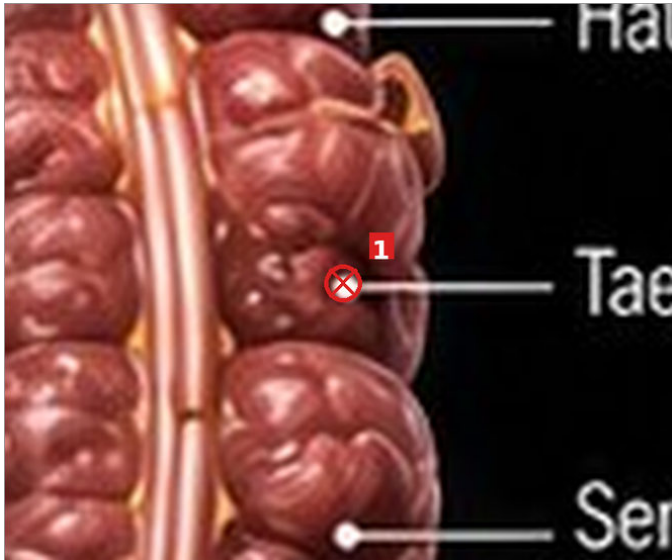
3 2 - Inferior mesenteric a. (VASCULAR SUPPLY panel) [CHECK]

NOW (941,589) the same pale longitudinal band, ~page(941,589), ~38px below leader 1

MOVE TO the red arterial trunk; note leader 3 ("Marginal artery of Drummond") lands on that band too, so three of the four leaders share one feature

diverticulitis-p2 — close-ups

red crossed circle = where it ends now green plain circle = where it belongs



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 -> the pale longitudinal band running vertically at
 ~page(834,220), about 35px to the left - the dot for
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diverticulitis-p2 — notes

NOTES

Rows 1-3 carry leader ink (13-18 px). Row 4 has no coordinate in the order at all - it is a whole-panel observation about the wall layers.